8.3 Diabetic Ketoacidosis

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I am very thirsty and have been vomiting." **Objective** • Glucose 480 mg/dL • Ketones positive, pH 7.15 • Kussmaul breathing • Mouth dry, HR 120/min	**ACTUAL** **Deficient fluid volume** related to osmotic diuresis and vomiting as evidenced by dry mucosa, tachycardia and hyperglycemia.	Within 6–8 hours, vital signs stable and urine output adequate.	• Give IV fluids as the DKA protocol prescribes. • Record urine output every hour, and I&O. • Vital signs and level of consciousness every hour. • Glucose every hour, electrolytes as ordered.	Hydrated and stable: **goal met**.
Subjective • "I feel I cannot catch my breath." **Objective** • Breathing deep and fast (Kussmaul) pH 7.15, HCO_3^- 10 mmol/L • Breath smells fruity	**ACTUAL** **Ineffective breathing pattern** related to compensatory response to metabolic acidosis as evidenced by Kussmaul respirations and low pH.	Within 24 hours, breathing pattern back to normal as the acidosis is corrected.	• Watch rate and depth of breathing and blood pH as ordered. • Give the insulin infusion and fluids according to the protocol. • Check potassium before and during insulin therapy and replace it as prescribed. • Watch the ECG for signs of a potassium problem.	Breathing and pH normal: **goal met**.
Subjective • *No specific complaint* **Objective** • On an insulin infusion	**RISK** **Risk for unstable blood glucose level** as evidenced by risk factors: IV insulin infusion.	Glucose comes down gradually, without a hypo.	• Check glucose every hour. • When glucose falls to the protocol level, add dextrose to the fluids. • Keep the insulin infusion running until ketones have cleared and subcutaneous insulin has been given.	No hypoglycemia: **goal met**.